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FEATURE ARTICLE

Architecture signifying social control: The restoration of asylumdom in mental health care?

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ABSTRACT: *Prior to the era of community care, the asylum has been an architectural manifestation of the power of psychiatry and the State. Asylumdom was a period in which custodial warehouses were used by the State to store sections of the population considered to be 'unreasonable'. There are signs, however, of a re-birth of asylumdom in both the UK and Australia. For example, there is a projected growth in the number of 'secure units' in the community, and a growing concern with issues of security and risk management in mental health care. Furthermore, new technologies for containment and surveillance are being installed in acute inpatient psychiatric units. Once again the asylum will symbolize emphatically the authority of the State (and its agencies of social control), and re-emphasize the exclusion of 'unreason' from the 'reasonable' society.*

KEY WORDS: *asylumdom, power, social control.*

INTRODUCTION

> 'The history of madness is the history of power ... It requires power to control it. Threatening the normal structures of authority, insanity is engaged in an endless dialogue ... about power.' (Porter, 1987: 39)

Throughout history, the powerful, whether regarded as a unitary and privileged class or as disconnected groupings dispersed throughout society, have attempted to maintain social order by exercising control over those deemed to be 'incomprehensible'. Over the last two millennia 'reason' has ascended as the overriding ideology. Rationality and reasonableness have formed the fundamental tenets of the industrial and technological epochs. Within this ideology, mental disorder becomes not merely a disease of the individual, but also a social malady. The 'health' of the whole of society becomes threatened by those without sanity.

It is the inexplicability and unpredictability of madness that intimidates the State. The bizarre thoughts and sensations of the psychotic, the impenitent constitution of the psychopath, the self-absorbed and self-destructive temperament of the neurotic, and the execrable conduct of the addict, all create alarm. In particular, however, it is aggressive, suicidal, and homicidal behaviour that reminds authority of the serious menace posed by the mad to social stability. Justified or

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not, media attention given to the latter towards the end of the twentieth century has provoked governments, and their agencies of social control, into actively defending the *status quo* (Morrall, 1999a).

ASYLUMDOM

The response of the powerful to those perceived to be *non compos mentis* has been one of social and/or physical apartheid. The building of hundreds of asylums in such countries as the UK, Australia and New Zealand was an ostentatious display of 'the power to expel' by the State and psychiatry.

Asylums, assembled beyond the scrutiny of the 'normal' population, had been in existence since the Middle Ages in Europe, but were to grow in number substantially during the nineteenth century (Shorter, 1997). Thousands of mentally disordered people were contained, from the seventeenth century onwards, within houses of correction, private mad houses, and local parish workhouses. They were caught up in the pan-European 'Great Confinement' (Foucault, 1971), which resulted in those who challenged the moral values of the bourgeois class being isolated from the rest of the population. These 'deviant' groups included the poor, the work-shy, the homeless and the criminals. In Foucault's (1971) terms, 'reason' was separated from 'unreason'.

Following the 1845 Lunacy Act in the UK local authorities were forced to provide for the mad through a massive public building program. Along with the Poor Law Amendment Act 1834, the 1845 Act heralded the beginning of 'Victorian Asylumdom' — the segregation of the mad into a centralized and unitary form of residence away from the rest of the community and other deviant populations (Foucault, 1971; Scull, 1979). Asylumdom represented a period in which custodial warehouses were used to 'store' sections of the population considered to be 'unreasonable' (Scull, 1993).

However, although asylumdom has been denigrated as an inhuman age due to the appalling incidents of ill-treatment (Gallagher, 1995), this was not necessarily the intention of those instigating the institutionalization of the mentally disordered:

> 'The lunacy reform movement of the early nineteenth century was driven forward, in substantial measure, by a utopian vision of the possibilities of asylum life.' (Scull, 1996: 8)

The imposing, and forbidding, architecture of the asylum for many of its inmates was a refuge from far worse conditions at the hands of their relatives. Edward Shorter recounts the testimony of Anton Muller, chief of psychiatry at the Royal Julius Hospital in Wurzburg, about the condition of newly admitted patients during the early part of the nineteenth century:

> 'A youth of sixteen, who for years had lain in a pigpen in the hut of his father, a shepherd, had so lost the use of his limbs and his mind that he would lap up the food from his bowl with his mouth just like an animal. When admitted to the hospital, Muller's patients ... were routinely found to have backs beaten blue, with bloody wounds. One man had been chained by his wife to the wall of their house for five years, losing the use of his legs.' (Shorter, 1997: 2–3)

While the Australian situation differs from that of the UK in a number of ways (e.g. there is no Australian equivalent of the Poor Law), the broad patterns of confinement of the insane found in the UK were also played out in the Australian colonies, and later in the States and Territories of the Commonwealth of Australia. Indeed, until recently, Australian mental health legislation has closely echoed concerns expressed (often more vigorously) in the UK, and Australian mental health reformers have looked to the 'old country' as a major source of new approaches to psychiatric care (Lewis, 1988).

The first instance of lunacy legislation in Australia was the Dangerous Lunatics Act of 1843 in New South Wales, which provided for safe custody and prevention of offences by persons considered dangerously insane. This Act, which was very much concerned with protecting the community against dangers posed by a person's insanity, established a number of procedures which became standard in subsequent Australian mental health legislation (e.g. certification by two

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medical practitioners, the appointment of official visitors) (Lewis, 1988). From the middle of the nineteenth century onwards, lunacy legislation was enacted in various Australian colonies: in South Australia in 1844 and 1864; Tasmania in 1858; Victoria in 1867; and Western Australia in 1871. Lewis (1988) suggests that the key principles of the Victorian lunacy law — regulation of private asylums, notification and admission of insane persons, official visits by commissioners, supervision by the medical profession, humane treatment for the insane, and administrative safeguards against wrongful confinement – were all present in the New South Wales Act of 1878.

The first 'psychiatric revolution' (Scull, 1981), occasioned by the beginning of asylumdom in countries such as the UK and Australia, presented the opportunity for the profession of medicine (and nursing) to emerge as legitimate 'surveyors' of the mad; that is, the medical profession was not instrumental in the creation of the asylum movement, but capitalized on a 'market opportunity':

> 'The sequesteration of lunatics was primarily an expression of civil policy, more an initiative from magistrates, philanthropists and families than an achievement — for good or ill — of the doctors. Indeed, the rise of psychological medicine was more the consequence than the cause of the insane asylum. Psychiatry could flourish once, but not before, large numbers of inmates were crowded into asylums.' (Porter, 1987: 17)

It was the role of the psychiatrist as administrator, rather than that of the skilled and effective practitioner, which provided the opportunity for a power base to be established in the asylum (Rogers & Pilgrim, 1996; Scull, 1993). From the later part of the eighteenth century, 'moral treatment', supported by lay philanthropists and religious groups such as the Quakers, competed with the stimulants, sedatives, emetics, purgatives, bloodletting, cold and hot baths, mechanical restraints, and electric shocks, of 'organic' psychiatry. Moral management invoked a more benevolent approach, which held that the mad could be brought back to reason if handled more humanely:

> 'This movement aimed in effect to revive the dormant humanity of the mad, by treating them as endowed with a residuum at least of normal emotions, still capable of excitation and training ... They needed to be treated essentially like children, who required a stiff dose of rigorous discipline, rectification and retraining in thinking and training.' (Porter, 1987: 19)

By the early nineteenth century, the medical profession in countries such as the UK, Australia and the USA, had gained legal control over asylums. Employing the tried and tested technique of 'inclusion', rather than attacking popular 'alternative' techniques, doctors embraced moral treatment within their assortment of procedures. The effect of 'medicalising' moral treatment was to leave psychiatrists '... in charge of the whole enterprise' (Johnstone, 1989). By the time the 1890 Lunacy Act was instituted in the UK (with similar Australian acts being passed in Western Australia and Victoria in 1903, and South Australia in 1913), the monopolization of the care of the mad by the medical profession enabled doctors to re-define the category of 'madness' to that of 'mental illness' (Baruch & Treacher, 1978). The medicalization of one form of deviancy (psychological disturbance) had thus been achieved.

DE-INSTITUTIONALIZATION

With the demise of the policy of incarcerating the mad, slum housing, prisons and the streets of the city provided alternative accommodation to be shared with other groups on the periphery of society. Numerous commentators have pointed to the ways in which 'care in the community' has been accompanied by an exponential upsurge in the ghettoization and neglect of the mentally ill (Barham, 1992; Isaac & Armat, 1990; Lamb, 1992). As various countries have moved to implement mental health care in the community, each has also experienced a substantial rise in the number of mentally ill people who live in squalor, are homeless, reside in bed and breakfast accommodation, are jailed, and who have joined the ranks of the long-term unemployed (Audit Commission, 1994; Bennetto, 1998; Craig,

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Bayliss, Klein, Manning & Reader 1995; Mihill, 1995; Hoult & Burchmore, 1994; Human Rights and Equal Opportunity Commission, 1993; Johnson *et al.*, 1997; Murphy, 1991). For all too many experiencing mental disorder, the price for 'social integration' has been profound social indifference. However, while finding themselves on the margins of society, the mentally ill have still attracted considerable media and (thus) public attention (Allen & Nairn, 1997; Hazelton, 1997a; Lupton, 1993). The violent tendencies of a minority of the most seriously mentally distressed have increased the visibility of 'unreason'.

In the UK, a plethora of reports from pressure groups, inquiries, and the media have expressed outrage about inadequate provision and, with some justification (Morrall, 1997a), the 'lack of rigour' by mental health professionals and managers of health services in executing control over the mentally ill:

> 'A series of errors were made in the care of a mental patient who went on to kill his neighbour, an independent report concluded yesterday...The report says [that the] death could not have been predicted. But it adds that [the patient] had received a poor standard of care, and vital information was not passed from one agency to another.' (unattributed editorial, 1998)

Similar concerns have been raised in the Australian context, focusing largely on media-led allegations that the deinstitutionalization of seriously mentally ill persons has resulted in inadequate care and supervision of those who might be a danger to themselves or others (Hazelton, 1997b). Similarly to the UK, it is a number of acts of extreme violence — especially the Port Arthur shootings — carried out in various parts of the country, which have served to sharpen public debate regarding the supposed 'crisis' in mental health services (McCallum, 1998).

RE-CARCERATION

The mentally ill have been at risk of 're-carceration' throughout the era of 'care in the community' (Morrall, 1987); that is, opposition to de-institutionalization has endured, with demands persisting for the Victorian asylums to be used again for the care of the mad. Furthermore, under the epithet of *'The Third Way'*, the government of the UK has launched its plan to renovate mental health provision. A cash injection of £700 million has been promised to accommodate a 'root and branch' overhaul of services. There is also to be a re-assessment of the whole of mental health law, and guidelines produced on 'national standards' (Department of Health, 1998). Although this is presented as a strategy to avoid the pitfalls of policies based solely on either the community or the asylum, there is in effect to be a re-emphasis on institutional care and public safety. There is much more weight given to acute inpatient hospital services, secure accommodation, and the need to protect both the public and the mentally ill by removing 'dangerous' patients from the community (as well as the use of 'assertive outreach teams' and the establishment of a national framework to provide guidance on expected levels of care), rather than being a pledge to re-invest in the policy of care in the community. There are also proposals to detain people with personality disorders on the basis of their assumed dangerousness (Home Office, 1999).

Similar trends are also being played out in Australia, and given the existence already of a National Mental Health Strategy (Australian Health Ministers, 1993), may even be further advanced. However, while the general thrust of mental health policy developments in Australia are decidedly progressive, these are nevertheless being accompanied by the development of what might be thought of as an ongoing (re-)routinization of surveillance and security technologies in mental health facilities throughout the country, including the use of security fences, high dependency units, closed-circuit television monitoring, and electronic security alarms for staff. Taken together, these old (i.e. security fences) and new (i.e. high dependency units) technologies (of containment and security) might usefully be thought of as a kind of new psychiatric institutionalism (Hazelton, 1999).

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PROGRESS

The asylum, it should be acknowledged, is an architectural and philosophical edifice of the modernist age. It exemplified the rationality of social engineering and scientific medicine. It was afterall, from the points of view of the Victorian eugenicist and bourgeois philanthropist, quite 'sensible' to protect society and the 'insensible' from each other. Psychiatry had a pre-packaged laboratory, with captive 'guinea-pigs' on whom therapeutic experimentation (whether 'moral', psychological, pharmaceutical, or surgical) could be indulged with impunity.

Moreover, the birth of the asylum was as a consequence of the State implementing centralized control measures (Scull, 1993). It was not until the nineteenth century that the State could benefit from capitalist administrative *savoir-fair*, a necessary pre-condition for the consolidation of social control measures. Furthermore, capitalist enterprise demanded economic and social rationalism, signified (in part) by the exclusion of the socially disruptive. Consequently, there is an inviolable link between capitalist expansion, the State, and such exclusionary apparatus as the asylum.

This modernism implies that the social organism is in a condition of perpetual progress. Education, science, and technology, are considered to be capable of producing a better world in which wars will have no purpose and therefore will not occur, affluence will be pandemic, and illness will be eradicated.

CHAOS

The world in the late twentieth century has been going through a series of tumultuous transformations as established social structures react to techno-scientific revolutions, a re-ordering of previous economic arrangements between societies, the creation of globalized markets and communications systems, and a metamorphosis in cultural values. As one influential commentator recently put it:

> 'We may no longer be living under the aegis of an industrial or capitalist culture which can tell us what is true, right, beautiful, and also what our place is in the grand scheme, but under a chaotic, mass-mediated, individual-preference-based culture of postmodernism.' (Waters, 1994: 206)

From this perspective, there are no *a priori* moral, fiscal, or political principles on which individual and social behaviour can be forged. In this revolutionary period, the institutions which nurtured human society in the past are dead, dying, or mutating. For example, there is no longer a perceived norm for the membership and organization of society's core social grouping — the family.

Political parties have jettisoned their historical ideological affiliations to particular sections of society in favour of 'pragmatic' ('rational') solutions to each event or issue. Workers rarely now have stable and long-term jobs, but face instead the prospect of vacillating career 'opportunities' and 'flexible' employment conditions. 'Authentic' health care now encompasses such disparate and epistemologically incompatible medical interventions as gene therapy, acupuncture, neural grafting, homeopathy, and psychoanalysis. Moreover, there are no assurances that these new social arrangements will themselves be anything more than ephemeral. The practices of the family, politicians, workers, and doctors may change in ways yet to be defined, or could revert to previous patterns. The postmodernist dictum, therefore, can be summed up as 'the only certainty is uncertainty'.

RELATIVE CONSTANCY

However, there is a pattern within the chaos. Whatever social, political or economic discordance exists in Western society, the system remains remarkably stable, and unremittingly committed to a capitalist mode of production. Moreover, although there have been serious threats to its dominance (e.g, through the growth of 'complementary' medicine, an increased interest in spirituality, the intellectual posturing of postmodern philosophy) scientific rationality persists as the core explanatory paradigm (Sokal & Bricmont, 1997). Capitalism has now a global

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market, and techno-scientific discoveries (e.g, transnational media networks) permeate virtually every culture in the world.

Furthermore, the ultimate function of psychiatry and psychiatric nursing, of operating on behalf of the State to prevent social ataxia, endures (Morrall, 1998). Regardless of professional and altruistic pretensions, medicine and nursing continue to monitor and regulate the form of deviancy (i.e. 'madness') that is most challenging to capitalist rationalism. The pressure on the psychiatric professions to 'discipline' the non-conformist has probably increased rather than diminished in the last decade as the postmodernism of the late twentieth century comes to understand itself as being in some form of constant crisis. As the following excerpt indicates, the medical profession's formal powers, to detain and/or forcibly administer treatment, undoubtedly remain vigorous:

> 'A hospital ... forcibly detained a sane woman in a psychiatric ward because she tried to starve herself to death ... a former teacher and professional diver suffered a series of strokes that left her unable to feed herself or to attend to her personal hygiene ... Doctors wanted to administer electro-convulsive therapy and forcibly re-hydrate her after she had stopped drinking.' (Illman, 1997).

Psychiatrists, as social control experts, are fundamental to the process of 'medicalizing' a loose array and growing number of socially discordant behaviours (Scull, 1993). In the fourth edition of the American Psychiatric Association's (1994) *Diagnostic and Statistical Manual of Mental Disorders*, there are 800 illness categories. These range from 'Alzheimer's disease' to 'Female Sexual Arousal Disorder'!

However, psychiatrists (and mental health nurses) exercise considerable discretionary power in their professional practice; psychiatry is relatively autonomous in its ability to constantly extend its practice without regard for the immediate exigencies of capitalist production. But it would be wrong to think of psychiatry (or any profession, including mental health nursing) as working directly to fulfil the functional requirements of either the capitalist state or capitalist production. Rather, under the conditions of liberal-democracy, governmental programs, such as mental health, must operate 'at a distance', and often through non-governmental agents and agencies (such as psychiatry and mental health nursing) to achieve their aims (Rose & Miller, 1992).

Nevertheless, it must be acknowledged that psychiatry continues to operate under State sanction; that is, psychiatry has been bestowed with the capacity to arbitrate, within a (relatively) pre-set framework, the boundaries between normality and abnormality. It can also promote vested interests (as it did in gaining authority to manage the asylums). But this does not detract from the overall State-sanctioned task of psychiatry to oversee those deemed to be socially disruptive, psychiatrically dangerous, or socially unacceptable.

CONCLUSION

Traditionally, the asylum has been the architectural manifestation of the power of psychiatry. There are now signs of a re-birth of asylumdom in the form of a projected growth in the number of 'secure units' in the community; a growing preoccupation with issues of risk management and security among mental health personnel and other mental health stakeholders, and a routinization of surveillance and security technologies, including security fences, high dependency units, closed-circuit television monitoring, and electronic personal security alarms, in mental health facilities in countries such as the UK and Australia. Once again the asylum (in different forms) will symbolize emphatically the authority of the State (and its agencies of social control), and re-emphasize the exclusion of 'unreason' from the 'reasonable' society. The re-emergence of asylumdom and its relationship to other influential discourses in mental health care — such as human rights discourse, managerial discourse and medical–psychiatric clinical discourses — surely warrants careful scrutiny by progressive mental health nurses and other mental health stakeholders.

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